



Stop Losing Patients to Voicemail

The Healthcare Practice Guide to AI Voice Agents

Start booking patients 24/7.

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The Problem Your Front Desk Can't Solve

The phone is still how patients reach you: 67% prefer to call. The average practice fields 53 calls per physician per day, and around 23% go unanswered. When a caller hits voicemail, 62% hang up without leaving a message, and 85% of people who cannot get through on the first try never call back. Four in ten calls arrive outside business hours, where a voicemail is where patients go to disappear.

1 in 4 patient calls goes unanswered

85% never call back after one try

41% of calls arrive after hours

Sources: MGMA, Dialog Health, ACC Solutions, industry composites, 2026.



The Real Cost of a Missed Call

Every unanswered call is a patient acquisition investment, \$200 to \$600, walking out the door, and the losses compound. No-shows alone cost the average practice \$22,872 a year, and dental practices lose more than \$100,000 per location to missed calls. Here is the monthly bleed for a mid-size practice.

These are not soft numbers. Each empty slot is worth \$200 or more, and a new patient you never reach is worth \$3,000 to \$6,000 over the life of the relationship. Multiply your own daily misses by your own new-patient value, and the annual figure usually lands somewhere most owners would rather not look. The phone, not the marketing budget, is where the money is leaking.

A practice missing 25 calls a day loses \$3,000 to \$5,000 in potential revenue, every single day.

Based on industry averages for a mid-size practice. Sources: Curogram, Tebra, AnswerNet.

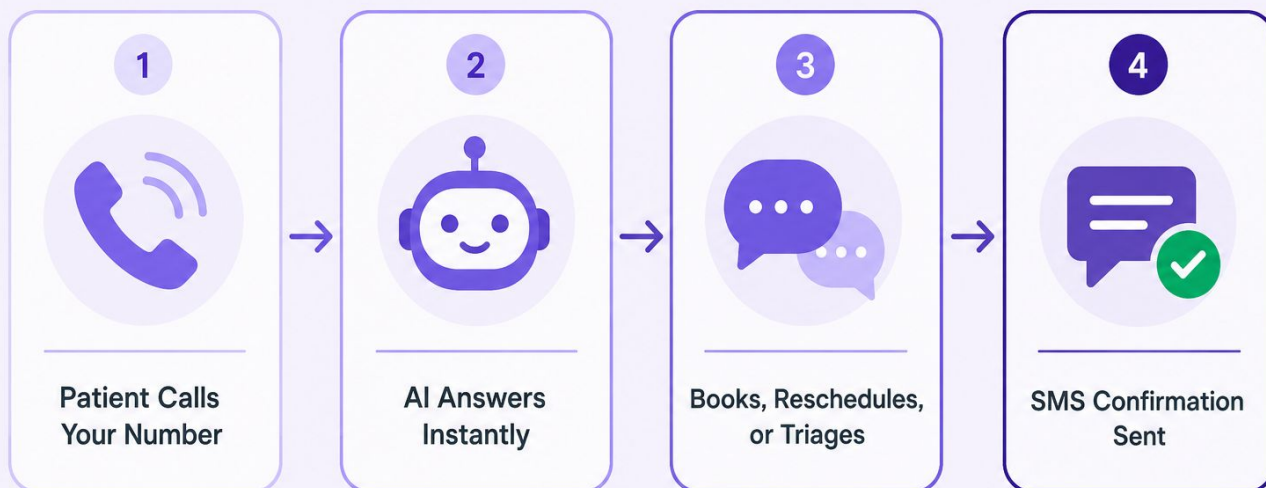
What Missed Calls Cost	Per Month
Missed calls per day	12
Value of a new patient	\$300
Monthly revenue lost	\$54,000+
Annual revenue lost	\$648,000+

What an AI Voice Agent Does

An AI voice agent answers your phone, has a natural conversation, and books, reschedules, or triages the patient, with no human needed. Most callers do not realise it is AI. It handles scheduling, after-hours capture, FAQs, and routing, 24/7 with zero hold time. It does not give medical advice, touch clinical records, or replace your clinical staff's judgement.

1. The patient calls your existing number.
2. The AI answers instantly, in under one second.
3. A natural conversation books, reschedules, or triages.
4. An SMS confirmation goes to the patient and the practice.

Available 24/7/365 with zero hold time.



Built for Your Kind of Practice

The same agent adapts to how your practice runs, from multi-provider scheduling to sensitive first-contact handling. Triage and routing stay administrative; anything clinical goes straight to your staff. Here is where it earns its keep across the main practice types.

The differences matter. A dental front desk needs same-day emergency triage and hygiene recall reactivation; a therapy practice needs an empathetic first contact and a clear crisis-routing rule; a veterinary clinic needs after-hours emergency routing and vaccination reminders.

The agent carries the same calm, capable manner into each, captures the few details that booking requires, and hands anything clinical to a human at once.

Crisis indicators route immediately to human resources, by design.

Practice Type	What the AI Handles
Medical and specialist	Multi-provider booking, after-hours triage, refill capture
Dental	Emergency triage, recall reactivation, insurance pre-checks
Therapy and counselling	Empathetic intake, therapist matching, crisis routing
Veterinary	Wellness and sick visits, emergency routing, reminders

The ROI Model for Your Practice

Modelled the way we model every SVA projection: conservative and transparent. Take a mid-size practice at 50 calls a day, answering 77% today, with no after-hours cover and a \$300 new-patient value. Answer every call, around the clock, and the maths looks like this.

The model is deliberately cautious. It assumes not every call is a bookable patient, that some missed calls would have been recovered on a callback anyway, and that booking still depends on having appointments free. Even after those haircuts, answering every call recovers six figures a year for a mid-size practice, from demand you are already paying to create. The same agent also trims no-shows by a third through automatic reminders.

What is one new patient worth to you over a lifetime, \$3,000, \$6,000, more? Now multiply by thirty extra bookings a month.

Conservative model. Individual results vary. No outcome is guaranteed.

Metric	Today	With AI Voice
Calls answered	77%	100%
After-hours capture	None	All of it
Booking rate	42%	55 to 65%
New bookings / mo	about 48	78 to 95
Revenue recovered / yr	Baseline	\$108K to \$169K

Compliance, Handled

AI voice in healthcare has to be compliant, and that is non-negotiable. In the US that means HIPAA; in Australia, the Privacy Act and its Australian Privacy Principles. Either way the rule is the same: protect patient information and collect only what you need.

SVA's approach is built for it. Every component in the voice stack is covered by a Business Associate Agreement, call data is encrypted in transit and at rest, and the agent captures scheduling details only. It never gives clinical advice, never touches your health records, and routes any clinical concern straight to your staff.

Penalties reach into the millions per year. General awareness only, not legal advice.



Compliance, Built In



Business Associate Agreement



Data Retention Policies



End-to-End Encryption



SOC 2 Type II



Role-Based Access Controls



No Clinical Advice by AI



Audit Logging



Minimal Patient Data by Design



What Patients Actually Experience

The conversation feels warm and human, because it is designed that way. The agent asks one thing at a time, never rushes, and confirms rather than interrogates. After hours, a caller gets the same calm, capable experience at 10pm as at 10am.

None of this is accidental. Each principle comes from SVA's Vapi Voice-AI Bible, the same playbook running in live production today. The agent trusts the data it has already captured instead of reading it back, skips the friction of spelling out an email by voice, and keeps the call short without ever sounding hurried. The result is a front desk that sounds like your best receptionist on her calmest day, at 2am, on a public holiday, every time.

'I didn't realise it wasn't a person until my husband told me.' (illustrative of typical reactions)

Design Principle	What the Caller Feels
One question per turn	Never bombarded, it flows naturally
Short but not rushed	Booked in under two minutes
Confirm, don't re-ask	No reading out ten digits
No email by voice	Confirmed by SMS instead
Minimum viable qualification	Only what the booking needs

Is This Right for Your Practice?

This guide is for you if any of these sound familiar. If you already answer 95% of calls with dedicated staff, or take fewer than ten calls a day, you may not need it yet.

1. Your front desk misses more than one in ten calls.
2. You lose patients to voicemail after hours or over lunch.
3. Your no-show rate is above 15%.
4. You spend \$200 or more to acquire each new patient.
5. Your staff lose two hours a day to routine scheduling calls.
6. You want 24/7 coverage without hiring night staff.

If you cannot say what your current call answer rate is, that is finding number one.

See What Your Practice Is Leaving on the Table

Book a free 15-minute practice revenue audit:

- 1** We analyse your call volume, missed-call rate, and no-show data.
- 2** You get a custom ROI projection for your practice.
- 3** No obligation, no pressure.

Trusted by practices across Australia, on HIPAA-ready, Privacy Act-aligned infrastructure.

Want to hear it first? Ask us for a sample call of the AI handling a real scheduling conversation. Reply to the email that delivered this guide.



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